Part 3. Staffing Policies and Procedures

T3C-S1 Staffing of T3C Functions

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POLICY

Benevolent House assigns every staffing function required for its Service Packages and Add-On Service to a named employee or a contracted provider, and keeps the Organizational Chart and this packet current.

PROCEDURE

1. A staff member may serve more than one function. Case Management and Treatment Director functions are performed only by employees of Benevolent House.
2. Each person meets the qualifications for every function the person performs. The Executive Director verifies qualifications at hire and whenever a function is reassigned, and keeps the documentation in the personnel record.
3. The Program Director and the Executive Director cover for one another, and each case-carrying staff member covers for the others.
4. For a child on the Program Director's own caseload, the Executive Director approves the admission and signs the continued stay confirmation in the Program Director's place.
5. A contracted provider is trained in, practices, and stays current in the Treatment Model, and is overseen by the staff member named in Part 1.
6. The Executive Director updates the Organizational Chart and Part 1 of this packet when an assignment changes.

T3C-S2 Workload Review

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POLICY

Benevolent House regularly reviews workloads so that the number of children or homes each person serves is appropriate to the Treatment Model, the Service Package, and the complexity of the cases. The staff to child ratios in the T3C System Blueprint are used as the reference point.

PROCEDURE

Function	Blueprint reference ratio	Applies to
Case Management	1 to 20 children	Basic
Case Management	1 to 15 children	Mental & Behavioral Health
Licensed Therapist	1 to 14 children	Mental & Behavioral Health
Behavior Support Specialist / Mentor	1 to 15 children	Mental & Behavioral Health
Crisis Management	1 to 25 children	Mental & Behavioral Health
Aftercare Case Management	1 to 25 children	Mental & Behavioral Health
Kinship Caregiver Home Support	1 to 7 kinship homes	Kinship Add-On
Aftercare Kinship Support	1 to 25 kinship homes	Kinship Add-On

1. **When.** The Executive Director and the Program Director review workloads each quarter.
2. **What is reviewed.** For each person: the number of children by Service Package, the number of kinship homes and aftercare cases, and the other functions the person performs.
3. **Case complexity.** The review weighs the needs of the children served, sibling groups, the number of homes, travel, children new to placement, and children at risk of a placement change.

4. **Signs of strain.** The review looks at whether Service Plan reviews, monthly visits, written confirmations, aftercare contacts, and documentation were completed on time.
5. **Action.** When a workload is above the reference ratio, or work is not being completed on time, the directors take one or more of these steps: reassign children or functions among staff, hold new admissions to the affected Service Package, or add a case manager.
6. **Contracted therapist.** The Treatment Director confirms each quarter that the contracted provider has the capacity to serve the children referred, and arranges another STAR Health provider when it does not.
7. **Record.** The review and any action taken are recorded in the Continuous Quality Improvement minutes.

T3C-S3 Policies and Procedures for Each Function

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POLICY

Each T3C function is carried out according to the policies and procedures listed below. Policies numbered T3C-1 through T3C-13 are in the T3C Service Delivery Addendum to the Policy and Operations Manual.

PROCEDURE

Function	Governing policies and procedures
Licensed Administrator; Program Director	T3C-1 Scope and Authority; T3C-4 Admission Guidelines; T3C-6 Continued Stay; T3C-10 Home Credentialing; T3C-S1 and T3C-S2 in this packet; Policy and Operations Manual Section IV.
Treatment Director	T3C-2 Treatment Model Integration; T3C-5 Service Planning; T3C-6 Continued Stay; T3C-S4 in this packet.
Case Management	T3C-2, T3C-3, T3C-5, T3C-6, T3C-7, T3C-8, and T3C-13; T3C-S2 in this packet; Policy and Operations Manual Section VII, Service Planning and Discharge.
Intake / Placement	T3C-4 Admission Guidelines; Policy and Operations Manual Section VI, Admissions and Placement.
Staff Training and Workforce Development	T3C Training Curriculum and Plan; T3C Policy and Procedure Training; Part 4 of this packet; Policy and Operations Manual Section V.
Caregiver Recruitment and Retention	T3C-10 Home Credentialing; T3C-12 Paid Intermittent Alternate Care; Policy and Operations Manual Section XI, Screening and Verification.
Staff Recruitment and Retention	T3C-S1 and T3C-S2 in this packet; Policy and Operations Manual Section IV.
Education Liaison; Cross-System Coordination	T3C-5, Sections D and E.
Continuous Quality Assurance and Improvement	T3C Logic Models and Continuous Quality Improvement Plan; T3C-6, Section D.
Billing, Cost Reporting, and Claims Administration	T3C-12 Paid Intermittent Alternate Care; T3C-S4 in this packet.
Licensed Therapist	T3C-2, Sections D and G; T3C-5, Section D; T3C-S2 and T3C-S4 in this packet.
Crisis Management; Behavior Support Specialist / Mentor	T3C-2, Section E; T3C-5, Sections A and F; T3C-S2 and T3C-S5 in this packet.
Aftercare Case Management	T3C-9 Aftercare Services; T3C-S2 in this packet.
Kinship Caregiver Home Support; Aftercare Kinship Support	T3C-11 Kinship Caregiver Support Services Add-On; T3C-9 Aftercare Services; T3C-S2 in this packet.

T3C-S4 Oversight of Contracted Functions

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POLICY

A function provided under contract is held to the same requirements as a function performed by staff.

PROCEDURE

1. Each contracted function is covered by a signed written agreement kept on file.
2. The Treatment Director oversees the Licensed Therapist agreement, verifies each provider's license, and confirms the provider has completed the TST-FC orientation before the provider gives oversight of treatment and service planning.
3. The Program Director oversees the billing agreement and reviews billing each month for the correct Service Package, Add-On Service, foster family pass-through, and paid Intermittent Alternate Care.
4. The overseeing staff member raises any concern with the contractor in writing and reports unresolved concerns to the Executive Director.

T3C-S5 Crisis Management and On-Call Response

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POLICY

Case manager level or above staff are available 24 hours a day, 7 days a week, to respond to any crisis involving a child in the Mental & Behavioral Health Support Services Package and to give an immediate response to kinship caregivers.

PROCEDURE

1. Every caregiver is given the on-call number at placement.
2. On-call staff respond by phone, by video conference, or in person, according to what the situation requires.
3. On-call staff have access to each child's safety and crisis plan and Managing Emotions Guide.
4. Serious incidents are reported and documented as required by Minimum Standards and the Policy and Operations Manual.
5. The case manager follows up with the caregiver after the crisis as described in Policy T3C-2, Section E.